

the underlying cause to [JB63.5](#) *Other viral diseases complicating pregnancy, childbirth or the puerperium*. For greater specificity, also add the code for [1D2Z](#) *Dengue, unspecified* Dengue with warning signs to the cluster. ([JB63.5](#) / [1D2Z](#))

Example 7

- 1 (a) Disseminated intravascular coagulation [3B20](#)
(b) Obstetric hemorrhage [JA4Z](#)
(c) Postpartum uterine atony [JA43.1](#)
(d) Abruptio placentae [JA8C.Z](#)

2

The deceased was pregnant: Yes, within 42 days of death; Pregnancy contributed to death: Yes

All DIC, obstetric hemorrhage, and postpartum uterine atony can be caused by Abruptio placentae [JA8C.Z](#) and that is the tentative starting point. As DIC is reported and this gives greater specificity apply Step M2 and code to [JA8C.0](#) Premature separation of placenta with coagulation defect. As no other special instructions apply, and the death was within 42 days after the obstetric cause, [JA8C.0](#) is the underlying cause of death.

Extraction and examples of special instructions on maternal mortality

Followings are special instructions specific to categories in Chapter 18. For instructions of each step refer to corresponding sections.

SP3 & SP4 Accepted and rejected sequences of maternal mortality (see Section [2.17.6](#), [2.17.7](#) and [2.19.1.14](#))

Do not accept Ectopic pregnancy ([JA01.-](#)) and Molar pregnancy ([JA02.-](#)) as due to other causes.

Consequence condition Causal condition

[JA01.-](#) Ectopic pregnancy **Do not accept** other causes

[JA02.-](#) Molar pregnancy

Do not accept Hypertensive disorders in pregnancy, childbirth, or the puerperium ([JA20-JA21](#), [JA23-JA25](#)) as due to other causes.

Consequence condition

[JA20-JA21](#), [JA23-JA25](#) Hypertensive disorders in pregnancy, childbirth, or the puerperium

Causal condition

Do not accept other causes

Do not accept Maternal care related to placenta praevia or low lying placenta ([JA8B.-](#)) as due to other causes.

Consequence condition

[JA8B.-](#) Maternal care related to placenta praevia or low lying placenta

Causal condition

Do not accept other causes

Example 8

- | | | | |
|---|---------------------------|--------|------------------------|
| 1 | (a) Hypovolemic shock | 1 day | MG40.1 |
| | (b) Obstetric haemorrhage | 1 day | JA4Z |
| | (c) Placenta praevia | 1 day | JA8B.1 |
| | (d) Pneumonia | 4 days | CA40.Z |

2

The deceased was pregnant: Yes, at the time of death; Pregnancy contributed to death: Yes

Placenta praevia cannot be caused by pneumonia and this sequence is rejected by the special instructions on the accepted and rejected sequence. There is an acceptable sequence which is hypovolemic shock caused by obstetric haemorrhage, in its turn caused by placenta praevia, and [JA8B.1](#) is the tentative starting point (Step SP4). As no other special instructions apply, and the death was within 42 days after the obstetric cause, [JA8B.1](#) is the underlying cause of death.

SP5 - Terminal cause of death when no sequence (See Section [2.17.8](#))

Example 9

- | | | |
|---|-----------------------------|------------------------|
| 1 | (a) Amniotic fluid embolism | JB42.1 |
| | (b) | |
| | (c) | |
| | (d) | |

2

There is no sequencing ending with the terminal cause of death in Part one. Amniotic fluid embolism [JB42.1](#) is the tentative starting point (Step SP5), and as no other special instructions apply it is also the underlying cause of death.

SP8 - Conditions unlikely to cause death of maternal mortality (See Section [2.17.11](#) and Mortality Annex [3.14](#))

Code	Title
JA65.3	Low weight gain in pregnancy
JA65.4	Pregnancy care of habitual aborter
JA66 .-	Clinical findings on antenatal screening of mother
JA8D .-	Maternal care related to false labour
JB00.0	Preterm labour without delivery
JB46.0	Retracted nipple associated with childbirth
JB46.2	Other and unspecified disorders of breast associated with childbirth
JB46.3	Agalactia
JB46.4	Hypogalactia
JB46.5	Suppressed lactation
JB46.6	Galactorrhea
JB46.7	Other and unspecified disorders of lactation

Example 10

1	(a) Galactorrhoea in puerperium	10 days	JB46.6
	(b)		
	(c)		
	(d)		
2	Puerperal sepsis after cesarean section	6 days	JB40.0

Galactorrhoea in puerperium [JB46.6](#) is the tentative starting point (Step SP5), but it is in the 'List of conditions considered unlikely to cause death'. There is another condition on the certificate, Puerperal sepsis after cesarean section, which is not in the 'List of conditions considered unlikely to cause death'. Disregard galactorrhoea in puerperium by Step SP8 and restart the selection procedure from Step SP1. Puerperal sepsis after cesarean section is selected as the underlying cause of death.

M1 – If the conditions specified in the right-hand column apply, then use the code in bold as the new tentative underlying cause (See Section [2.18.1](#) and [2.19.3](#))

TUC is with mention of: code to:
[JA24](#).- Pre-eclampsia [JA25](#).- Eclampsia [JA25](#).-

<i>TUC is</i>	when reported as the cause of:	code to:
JA61.0 Varicose veins of lower extremity in pregnancy	JB42.2 Obstetric blood-clot embolism	JB42.2
JA61.1 Genital varices in pregnancy		
JA61.2 Superficial thrombophlebitis in pregnancy		
JA61.4 Haemorrhoids in pregnancy		
<i>TUC is</i>	when reported as the cause of:	code to:
JA65.2 Excessive weight gain in pregnancy	JA20 - JA20-JA2Z Oedema, proteinuria, or hypertensive disorders in pregnancy, childbirth, or the puerperium	JA20 - JA20-JA2Z
<i>TUC is</i>	with mention of:	code to:
JA65.3 Low weight gain in pregnancy	JB64.2 Endocrine, nutritional or metabolic diseases complicating pregnancy, childbirth or the puerperium	JB64.2
<i>TUC is</i>	when reported as the cause of:	code to:
JA65.4 Pregnancy care of habitual aborter	JA00 - JA0Z Abortive outcome of pregnancy	JA00 - JA0Z
<i>TUC is</i>	when reported as the cause of:	code to:
JA65.5 Retained intrauterine contraceptive device in pregnancy	JA00 - JA0Z Abortive outcome of pregnancy	JA00 - JA0Z
	JA88.1 Infection of amniotic sac and membranes	JA88.1
	JB00 .- Preterm labour or delivery	JB00 .-
<i>TUC is</i>	with mention of:	code to:
JA65.7 Subluxation of symphysis pubis in pregnancy, childbirth or the puerperium	JB04 - JB06 Obstructed labour due to malposition or malpresentation of fetus, maternal pelvic abnormality or due to other causes	JB04 - JB06

<i>TUC is</i>	when reported as the cause of:	code to:
JA67.4 Spinal and epidural anaesthesia-induced headache during pregnancy	JA67.2 Central nervous system complications of anaesthesia during pregnancy	JA67.2

<i>TUC is</i>	with mention of:	code to:
JA80 .- Maternal care related to multiple gestation	JB04 - JB06 Obstructed labour due to malposition or malpresentation of foetus, maternal pelvic abnormality, or other causes	JB04 - JB06
JA81 .- Maternal care related to complications specific to multiple gestation	JB0D.3 Other complications of obstetric surgery or procedures	JB0D.3
	JB42.1 Amniotic fluid embolism	JB42.1

<i>TUC is</i>	when reported as the cause of:	code to:
JA82 .- Maternal care for known or suspected malpresentation of fetus	JA43 .- Postpartum haemorrhage	JA43 .-
JA83 .- Maternal care for known or suspected disproportion	JB04 - JB06 Obstructed labour due to malposition or malpresentation of foetus, maternal pelvic abnormality, or other causes	JB04 - JB06
JA84 .- Maternal care for known or suspected abnormality of pelvic organs	JB09 - JB0A Perineal laceration during delivery and other obstetric trauma	JB09 - JB0A
JA85 .- Maternal care for known or suspected fetal abnormality or damage	JB0D .- Certain specified complications of labour or delivery, not elsewhere classified	JB0D .-
JA86 .- Maternal care for other known or suspected fetal problems	JB40 .- Infections in the puerperium	JB40

<i>TUC is</i>	with mention of:	code to:
JA82 .- Maternal care for known or suspected malpresentation of fetus	JA83 .- Maternal care for known or suspected disproportion	JA83 .-

<i>TUC is</i>	with mention of:	code to:
JA83.7 Maternal care for known or suspected disproportion, unspecified	JA83.0 Maternal care for disproportion due to deformity of maternal pelvic bones	JA83.0
	JA83.1 Maternal care for disproportion due to generally contracted pelvis	JA83.1
	JA83.2 Maternal care for disproportion due to inlet contraction of pelvis	JA83.2
	JA83.3 Maternal care for disproportion due to outlet contraction of pelvis	JA83.3

<i>TUC is</i>	when reported as the cause of:	code to:
JA87.- Maternal care related to polyhydramnios	JA8C.- Maternal care related to premature separation of placenta	JA8C.-
	JB03.- Long labour	JB03.-
	JB04 - JB06 Obstructed labour due to malposition or malpresentation of foetus, maternal pelvic abnormality, or other causes	JB04 - JB06
	JA43.- Postpartum haemorrhage	JA43.-
	JB0D.- Certain specified complications of labour or delivery, not elsewhere classified	JB0D.-
	JB40.- Infections in the puerperium	JB40.-
	JB42.1 Amniotic fluid embolism	JB42.1

<i>TUC is</i>	when reported as the cause of:	code to:
JA88.0 Oligohydramnios	JA43.- Postpartum haemorrhage	JA43.-
JA88.Y Other specified disorders of amniotic fluid and membranes	JA88.1 Infection of amniotic sac or membranes	JA88.1
JA88.Z Disorders of amniotic fluid and membranes, unspecified	JB03.- Long labour	JB03.-
	JB40.- Infections in the puerperium	JB40.-
	JB0D.- Certain specified complications of labour or delivery, not elsewhere classified	JB0D.-

<i>TUC is</i>	with mention of:	code to:
JA89.3 Premature rupture of membranes	JA88.1 Infection of amniotic sac or membranes	JA88.1
	JB03.- Long labour	JB03.-
	JA43.- Postpartum haemorrhage	JA43.-
	JB0D.- Certain specified complications of labour or delivery, not elsewhere classified	JB0D.-
	JB40.- Infections in the puerperium	JB40.-
	JB42.1 Amniotic fluid embolism	JB42.1

<i>TUC is</i>	when reported as the cause of:	code to:
JA8A.0 Placental transfusion syndromes	JA8C.- Maternal care related to premature separation of placenta	JA8C.-
JA8A.1 Malformation of placenta	JA42.- Intrapartum haemorrhage	JA42.-
JA8A.Y Other specified maternal care related to placental disorders	JA43.- Postpartum haemorrhage	JA43.-
JA8A.Z Maternal care related to placental disorders, unspecified	JB0D.- Certain specified complications of labour or delivery, not elsewhere classified	JB0D.-
	JB40.- Infections in the puerperium	JB40.-

<i>TUC is</i>	when reported as the cause of:	code to:
JA8A.2 Morbidly adherent placenta	JA8B.1 Placenta praevia with haemorrhage	JA8B.1
	JA8B.0 Placenta praevia specified as without haemorrhage	JA8B.0
	JA8B.Z Maternal care related to placenta praevia or low lying placenta, unspecified	JA8B.Z

<i>TUC is</i>	when reported as the cause of:	code to:
JA8E Maternal care related to prolonged pregnancy	JB0D.- Certain specified complications of labour or delivery, not elsewhere classified	JB0D.-

<i>TUC is</i>	when reported as the cause of:	code to:
JB00 .- Preterm labour or delivery	JA8C .- Maternal care related to premature separation of placenta	JA8C .-
JB01 .- Failed induction of labour	JA42 .- Intrapartum haemorrhage	JA42 .-
JB02 .- Abnormalities of forces of labour	JA43 .- Postpartum haemorrhage	JA43 .-
JB03 .- Long labour	JB04 - JB06 Obstructed labour due to malposition or malpresentation of foetus, maternal pelvic abnormality, or other causes	JB04 - JB06
JB07 .- Labour or delivery complicated by foetal distress	JB0D .- Certain specified complications of labour or delivery, not elsewhere classified	JB0D .-
JB08 .- Labour or delivery complicated by umbilical cord complications	JB40 .- Infections in the puerperium	JB40 .-

<i>TUC is</i>	with mention of:	code to:
JB04 .- Obstructed labour due to malposition or malpresentation of fetus	JB05 .- Obstructed labour due to maternal pelvic abnormality	JB05 .-

<i>TUC is</i>	when reported as the cause of:	code to:
JB09.2 , JB09.3 , JB09.Z Third, fourth or unspecified degree perineal laceration during delivery	JA43 .- Postpartum haemorrhage	JA43 .-
	JB40 .- Infections in the puerperium	JB40 .-

<i>TUC is</i>	when reported as the cause of:	code to:
JB0C.5 Spinal and epidural anaesthesia-induced headache during labour and delivery	JB0C.3 Central nervous system complications of anaesthesia during labour or delivery	JB0C.3

<i>TUC is</i>	when reported as the cause of:	code to:
JB0D.0 Maternal distress during labour and delivery	JA43 Postpartum haemorrhage	JA43
JB0D.1 Shock during or following labour and delivery	JB40 Infections in the puerperium	JB40
<i>TUC is</i>	when reported as the cause of:	code to:
JB0D.2 Pyrexia during labour, not elsewhere classified	JB40 Infections in the puerperium	JB40
<i>TUC is</i>	when reported as the cause of:	code to:
JB0D.4 Delayed delivery after artificial rupture of membranes	JA43.- Postpartum haemorrhage	JA43.-
JB0D.5 Delayed delivery after spontaneous or unspecified rupture of membranes	JB0D.- Certain specified complications of labour or delivery, not elsewhere classified	JB0D.-
	JB40.- Infections in the puerperium	JB40.-
<i>TUC is</i>	when reported as the cause of:	code to:
JB0D.6 Vaginal delivery following previous caesarean section	JA8A.2 Morbidly adherent placenta	JA8A.2
	JB0A.1 Rupture of uterus during labour	JB0A.1
	JB0A.2 Postpartum inversion of uterus	JB0A.2
	JA43.- Postpartum haemorrhage	JA43.-
	JB0D.- Certain specified complications of labour or delivery, not elsewhere classified	JB0D.-
	JB40.- Infections in the puerperium	JB40.-

<i>TUC is</i>	when reported as the cause of:	code to:
JB41.0 Superficial thrombophlebitis in the puerperium	JB42.2 Obstetric blood-clot embolism	JB42.2
JB41.2 Haemorrhoids in the puerperium		
JB41.Y Other venous complications in the puerperium		
JB41.Z Venous complication in the puerperium, unspecified		

<i>TUC is</i>	when reported as the cause of:	code to:
JB43.3 Spinal and epidural anaesthesia-induced headache during the puerperium	JB43.2 Central nervous system complications of anaesthesia during the puerperium	JB43.2

M1 - Conditions of maternal mortality not to be used as UCOD (See Section [2.19.3.24](#) for details)

TUC is:

JA05 .- Complications following abortion, ectopic or molar pregnancy	Code to: Abortive outcome of pregnancy of JA00-JA04
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TUC is:

JB65 Sequelae of complication of pregnancy, childbirth or the puerperium	Code to: JB62 .- Death from sequelae of obstetric causes
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M1 - Conditions of maternal mortality not to be used as UCOD under certain condition (See Section [2.18.1](#) and [2.19.3.18](#))

TUC is:

JA80 Maternal care related to multiple gestation	Not to be used for the underlying cause of death, if a more specific complication is reported. Code to: the more specific complication. If there is no specific complication, code to JB0Z Complications of labor or delivery, unspecified
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TUC is:

[JB20](#) - [JB2Z](#)
Delivery

Not to be used for the underlying cause of death, if a more specific complication is reported.

Code to: the more specific complication of [JB0C](#).- to [JB0D](#).-. If no complication is reported **code to** [JB0Z](#).

Example 11

- 1 (a) Subarachnoid haemorrhage 9 days [8B01.2](#)
(b) Eclampsia during pregnancy 10 days [JA25.0](#)
(c) Severe pre-eclampsia 11 days [JA24.1](#)
(d)
- 2

The deceased was pregnant: Yes, at the time of death; Pregnancy contributed to death: Yes

Severe pre-eclampsia is the tentative underlying cause according to Step SP3. There is a special instruction on 'Pre-eclampsia' reported with mention of 'eclampsia'. Apply this instruction and select [JA25.0](#) as the tentative underlying cause. As no other special instructions apply, and the death was within 42 days after the obstetric cause, [JA25.0](#) is the underlying cause of death.

M2 - Specificity (See Section [2.18.2](#))

Example 12

- 1 (a) Heart failure 1 day [BD1Z](#)
(b) Hypovolemic shock 2 days [MG40.1](#)
(c) Fourth degree perineal laceration 2 days [JB09.3](#)
(d) Perineal laceration during delivery 2 days [JB09.Z](#)
- 2

The deceased was pregnant: Yes, within 42 days of death; Pregnancy contributed to death: Yes

Perineal laceration during delivery ([JB09.Z](#)) is the tentative starting point by Step SP3. However, there is a category that provides more precise information about this condition which is fourth degree perineal laceration ([JB09.3](#)). Apply Step M2 and select [JB09.3](#) as the underlying cause of death.

M4 - Special instructions on surgery and other medical procedures (See Section [2.18.4](#) and [2.19.4](#))

Example 13

- 1 (a) Postoperative haemorrhage [NE81.0Z](#)
(b) Caesarean section [JB22.Z](#)
(c)
(d)

2

The deceased was pregnant: Yes, at the time of death; Pregnancy contributed to death: Yes
Reason for surgery: Prolonged labour

Caesarean section [JB22.Z](#) is the tentative by Step SP3. And the reason why the caesarean section was performed was stated as prolonged labour. Code [JB03.Z](#) Long labour, unspecified as the underlying cause of death.

Example 14

- 1 (a) Septic shock 1 day [1G41](#)
(b) Postpartum cardiomyopathy 1 month [JB44.3](#)
(c) Emergency caesarean section 1 month [JB22.1](#)
(d)

2

The deceased was pregnant: Yes, within 42 days of death; Pregnancy contributed to death: Yes

Emergency caesarean section [JB22.1](#) is the tentative underlying cause by Step SP3. The reason for caesarean section is not reported but the complication is. Code the complication, postpartum cardiomyopathy [JB44.3](#) as the underlying cause of death.

M4 – Special instructions on maternal mortality (See Section [2.19.7](#))

Terms used here in the instruction is as follows:

- Certain maternal diseases: [JA00.-](#) to [JB4Z](#), [JB60](#), [JB6Y](#), 1C14
- Maternal diseases classified elsewhere: [JB63](#) (infections), [JB64](#) (other)
- Injury or external causes: Chapter 22 or Chapter 23
- Other conditions: other than above

Maternal diseases - If the tentative underlying cause (TUC) is 'certain maternal diseases' and the deceased was pregnant at the time of death, within 42 days before death, or the duration is unknown or unstated, keep the TUC. - If the TUC is 'maternal diseases classified elsewhere' keep the TUC and postcoordinate the code for the specific disease classified elsewhere from Chapter 01-19. - If the TUC is [JB61.0](#), [JB61.Z](#), [JB62.0](#) or [JB62.Z](#), keep it and its post-coordinated code, if any, as the TUC. - If the TUC is 'certain maternal diseases' or maternal diseases classified elsewhere' but the deceased was pregnant in more than 42 days but less than one year before death, then code to [JB61.-](#) as the TUC and add the code for the specific maternal disease to the cluster.

Other conditions or indirect obstetric causes - If the TUC is [JB61.1](#) or [JB62.1](#), keep it and its post-coordinated code if any as the TUC. - If the TUC is 'other conditions', and the pregnancy contributed to death, and the deceased was pregnant at the time of death, within 42 days before death, or the duration is unknown or unstated, code to [JB63](#).- or [JB64](#).- as appropriate and add the specific 'other condition' to the cluster. - If the TUC is 'other conditions', and the pregnancy did not contribute to death, but the deceased was pregnant at the time of death, within 42 days before death, keep the TUC and add [XT0S](#) Pregnancy to the cluster. - If the TUC is 'other conditions', and the pregnancy did not contribute to death, and the deceased was pregnant in more than 42 days before death, or the duration is unknown or unstated, keep the TUC.

See 'Determining whether pregnancy contributed to death' in section 2.21.8.2 to decide whether pregnancy contributed to death from the information on the death certificate.

Injury or external causes - If the TUC is 'injury or external cause', and the deceased was pregnant at the time of death, within 42 days before death, keep the TUC and add XT03 Pregnancy to the cluster. - If the TUC is 'injury or external cause', and the deceased was pregnant in more than 42 days before death, or the duration is unknown or unstated, keep the TUC.

Workflow diagram for maternal mortality (See Section [3.14.4](#))

3.14.14 Target list of causes of death for verbal autopsy

The use of this list is two-fold. For computer-coding VA (CCVA) algorithms that assign broad text labels for causes of death, this list could serve as a coding list, such that the CCVA program could directly code the death to one of these labels, and this table provides the related ICD-11 codes. Alternatively, this list could serve as a tabulation list for other VA cause of death assignment methods such as physician coding or expert algorithms, which have the potential to directly assign specific text labels for causes of death with their individual ICD-11 codes. In such situations, the detailed coded data from these methods could then be aggregated for tabulation according to the code groups, to enable comparison with computer derived diagnosis. In other situations, the specifically ICD-11 coded data from physician coded VA or expert algorithms could be aggregated and analysed using other tabulation lists such as the WHO Mortality Lists or the Global Health Estimates/Global Burden of Disease categories. In all situations, coded data from VA should be specifically labelled according to the data source and the type of coding approach used, and separately tabulated for each data source/coding method. Column 1 contains the code for the verbal autopsy entity. Column 2 lists the related titles hyperlinked to the ICD-11 codes that would be used if the condition labelled by column 2 were coded to ICD-11.

Verbal autopsy code	Verbal autopsy title
VAs-01 Infectious and parasitic diseases	
VAs-01.01	Sepsis
VAs-01.02	Acute respiratory infection, including pneumonia
VAs-01.03	HIV/AIDS related death
VAs-01.04	Diarrheal diseases
VAs-01.05	Malaria
VAs-01.06	Measles
VAs-01.07	Meningitis and encephalitis
VAs-01.08	Tetanus
VAs-01.09	Pulmonary tuberculosis
VAs-01.10	Pertussis
VAs-01.11	Haemorrhagic fever
VAs-01.12	Dengue fever
VAs-01.13	Coronavirus disease (COVID-19)
VAs-01.99	Unspecified infectious disease
Non-communicable diseases	
VAs-98	Other and unspecified non-communicable disease
VAs-02 Neoplasms	
VAs-02.01	Oral neoplasms
VAs-02.02	Digestive neoplasms
VAs-02.03	Respiratory neoplasms
VAs-02.04	Breast neoplasms
VAs-02.05	Female reproductive neoplasms
VAs-02.06	Male reproductive neoplasms
VAs-02.99	Other and unspecified neoplasms
VAs-03 Nutritional and endocrine disorders	
VAs-03.01	Severe anaemia
VAs-03.02	Severe malnutrition
VAs-03.03	Diabetes mellitus
VAs-04 Diseases of the circulatory system	
VAs-04.01	Acute cardiac disease
VAs-04.02	Stroke
VAs-04.03	Sickle cell with crisis
VAs-04.99	Other and unspecified cardiac disease
VAs-05 Respiratory disorders	

Verbal autopsy code	Verbal autopsy title
VAs-05.01	Chronic obstructive pulmonary disease (COPD)
VAs-05.02	Asthma
VAs-06 Gastrointestinal disorders	
VAs-06.01	Acute abdomen
VAs-06.02	Liver cirrhosis
VAs-07 Renal disorders	
VAs-07.01	Renal failure
VAs-08 Mental and nervous system disorders	
VAs-08.01	Epilepsy
VAs-09 Pregnancy childbirth and puerperium-related disorders	
VAs-09.01	Ectopic pregnancy
VAs-09.02	Abortion-related death
VAs-09.03	Pregnancy-induced hypertension
VAs-09.04	Obstetric haemorrhage
VAs-09.05	Obstructed labour
VAs-09.06	Pregnancy-related sepsis
VAs-09.07	Anaemia of pregnancy
VAs-09.08	Ruptured uterus
VAs-09.99	Other and unspecified maternal cause
VAs-10 Neonatal causes of death	
VAs-10.01	Prematurity or low birth weight
VAs-10.02	Birth asphyxia
VAs-10.03	Neonatal pneumonia
VAs-10.04	Neonatal sepsis
VAs-10.05	Neonatal tetanus
VAs-10.06	Congenital malformation
VAs-10.99	Other and unspecified perinatal cause of death
VAs-11 Stillbirths	
VAs-11.01	Fresh stillbirth
VAs-11.02	Macerated stillbirth
VAs-12 External causes of death	
VAs-12.01	Road traffic accident
VAs-12.02	Other transport accident
VAs-12.03	Accidental fall

Verbal autopsy code	Verbal autopsy title
VAs-12.04	<u>Accidental drowning and submersion</u>
VAs-12.05	<u>Accidental exposure to smoke, fire and flames</u>
VAs-12.06	<u>Contact with venomous animals and plants</u>
VAs-12.07	<u>Accidental poisoning and exposure to noxious substance</u>
VAs-12.08	<u>Intentional self-harm</u>
VAs-12.09	<u>Assault</u>
VAs-12.10	<u>Exposure to force of nature</u>
VAs-12.99	<u>Other and unspecified external cause of death</u>
VAs-99	<u>Unknown and ill-defined cause of death</u>

3.15 Annex D: Differences between ICD-10 and ICD-11

3.15.1 Chapter 01 – Differences between ICD-10 and ICD-11 in Chapter 01

The chapter includes more infectious items than in the past. Also, influenza has been moved from the respiratory to the infectious diseases chapter. Tuberculosis, leprosy have been grouped under 'Mycobacterial diseases', because identification, course, and treatment are similar. Prion diseases have been moved to the Nervous system.

Table 1: Comparison of ICD-10 block structure with ICD-11 equivalent structure

ICD-10 block heading	ICD-11 equivalent structure
A00-A09 Intestinal infectious diseases	Gastroenteritis or colitis of infectious origin
A15-A19 Tuberculosis	Part of the grouping - Mycobacterial diseases
A20-28 Certain zoonotic bacterial diseases	Certain zoonotic bacterial diseases
A30-A49 Other bacterial diseases	Other bacterial diseases
A50-A64 Infections with a predominantly sexual mode of transmission	Predominantly sexually transmitted infections
A65-A69 Other spirochaetal diseases	Part of the grouping - Other specified bacterial diseases
A70-A74 Other diseases caused by chlamydiae	Part of the grouping – Other bacterial diseases
A75-A79 Rickettsioses	Part of the grouping – Other bacterial diseases
A80-A89 Viral infections of the central nervous system	Viral infections of the central nervous system
A92-A99 Arthropod-borne viral fevers and viral haemorrhagic fevers	Split into two groups - Other arthropod-borne viral fevers and Certain zoonotic viral diseases
B00-B09 Viral infections characterized by skin and mucous membrane lesions	Viral infections characterised by skin or mucous membrane lesions
B15-B19 Viral hepatitis	Viral hepatitis
B20-B24 Human immunodeficiency virus [HIV] disease	Human immunodeficiency virus disease
B25-B34 Other viral diseases	Certain other viral diseases
B35-B49 Mycoses	Mycoses
B50-B64 Protozoal diseases	Part of the grouping - Parasitic diseases
B65-B83 Helminthiasis	Part of the grouping - Parasitic diseases
B85-B89 Pediculosis, acariasis and other infestations	Part of the grouping - Parasitic diseases
B90-B94 Sequelae of infectious and parasitic diseases	Sequelae of infectious diseases
B95-B98 Bacterial, viral and other infectious agents	Now part of extension codes for organisms
B99-B99 Other infectious diseases	Certain other disorders of infectious origin

3.15.2 Differences between ICD-10 and ICD-11 in Chapter 02

The most significant change to the hierarchy of Chapter 02 is the inclusion of certain morphology types within the chapter (previously found in ICD-10, Appendix A). There are now precoordinated codes consisting of both morphology and site. Other types of morphology and greater site specificity not included in Chapter 02 are found in the Chapter X, Extension codes, and can be used for postcoordination.

Other changes include: grouping together all neoplasms of brain and central nervous system regardless of behaviour; grouping together all haematopoietic and lymphoid tissues; and the addition of the new group Malignant mesenchymal neoplasms. The previous ICD–10 group Neoplasms of uncertain or unknown behaviour has been split into two separate groups - Neoplasms of uncertain behaviour and Neoplasms of unknown behaviour.

Table 1: Comparison of ICD-10 block structure with ICD-11 equivalent structure

ICD-10 block heading	ICD-11 equivalent structure
C00-C97 Malignant neoplasms	Neoplasms of brain or central nervous system Neoplasms of haematopoietic or lymphoid tissues Malignant neoplasms, except of lymphoid, haematopoietic, central nervous system or related tissues
D00-D09 In situ neoplasms	In situ neoplasms, except of lymphoid, haematopoietic, central nervous system or related tissues
D10-D36 Benign neoplasms	Benign neoplasms, except of lymphoid, haematopoietic, central nervous system or related tissues
D37-D48 Neoplasms of uncertain or unknown behaviour	Neoplasms of uncertain behaviour, except of lymphoid, haematopoietic, central nervous system or related tissues Neoplasms of unknown behaviour, except of lymphoid, haematopoietic, central nervous system or related tissues

3.15.3 Differences between ICD–10 and ICD–11 in Chapter 03

ICD–10, Chapter 03 Disease of the blood and blood-forming organs and certain disorders involving the immune mechanism has been split into two chapters: one for diseases of blood or blood-forming organs (Ch. 03) and the other for disorders of the immune system (Ch. 04). In ICD-10 there were five major sections for blood disorders which have now been reclassified into three sections in ICD-11.

A broad grouping Anaemias and other erythrocyte disorders now contains, Nutritional anaemias, Haemolytic anaemias and Aplastic and other anaemias with subdivisions for acquired and congenital.

Table 1: Comparison of ICD-10 block structure with ICD-11 equivalent structure

ICD-10 block heading	ICD-11 equivalent structure
D50-D53 Nutritional anaemias	Part of the grouping - Anaemias and other erythrocyte disorders
D55-D59 Haemolytic anaemias	Part of the grouping - Anaemias and other erythrocyte disorders
D60-D64 Aplastic and other anaemias	Part of the grouping - Anaemias and other erythrocyte disorders
D65-D69 Coagulation defects, purpura and other haemorrhagic conditions	Coagulation defects, purpura and other haemorrhagic and related conditions
D70-D77 Other diseases of blood and blood-forming organs	Concepts redistributed to one of the following groupings: Anaemias and other erythrocyte disorders Coagulation defects, purpura and other haemorrhagic and related conditions or Diseases of spleen
D80-D89 Certain disorders involving the immune mechanism	Move to Chapter 04 'Diseases of the immune system'

3.15.4 Differences between ICD-10 and ICD-11 in Chapter 04

ICD-10, Chapter 03 'Diseases of the blood and blood-forming organs and certain disorders involving the immune mechanism' has been split into two chapters: one for diseases of blood or blood-forming organs (Ch. 03) and the other for disorders of the immune system (Ch. 04). This new chapter (Ch. 04) was created to better capture the complexity of the disease processes of the immune system.

Table 1: Comparison of ICD-10 block structure with ICD-11 equivalent structure

ICD-10 block heading	ICD-11 equivalent structure
D80-D89 Certain disorders involving the immune mechanism	Primary immunodeficiencies Acquired immunodeficiencies Autoinflammatory disorders Allergic or hypersensitivity conditions Immune system disorders involving white cell lineages Certain disorders involving the immune system Diseases of thymus

3.15.5 Differences between ICD-10 and ICD-11 in Chapter 05

Changes and additions have been made to Diabetes mellitus with the inclusion of categories for Intermediate hyperglycaemia (including Impaired glucose regulation) and Insulin-resistance syndromes. Nutritional disorders section incorporates current terminology and contains a detailed classification for vitamin and mineral deficiencies as well as for obesity.

The Metabolic disorders section also includes more detail and the organisation of the various types of metabolic disorders has been improved.

Table 1: *Comparison of ICD-10 block structure with ICD-11 equivalent structure in Chapter 5*

ICD-10 block heading

E00-E07 Disorders of thyroid gland

E10-E14 Diabetes mellitus

E15-E16 Other disorders of glucose regulation and pancreatic internal secretion

E20-E35 Disorders of other endocrine glands

E40-E46 Malnutrition

E50-E64 Other nutritional deficiencies

E65-E68 Obesity and other hyperalimentation

E70-E90 Metabolic disorders

ICD-11 equivalent structure

Disorders of the thyroid gland or thyroid hormones system. The structure for this section has not changed but has been revised to better reflect current disease processes.

Diabetes mellitus (Specifies the 'type' of diabetes mellitus i.e. Type 1, Type 2, other and unspecified. 'Diabetic' complications are primarily parented to their respective body system chapter).

Other disorders of glucose regulation or pancreatic internal secretion – has remained unchanged.

This block has been unbundled and the sections renamed to better reflect the conditions classified within each entity:

Disorders of the parathyroid and parathyroid hormone system

Disorders of the pituitary hormone system

Disorders of the adrenal glands and adrenal hormone system

Disorders of the gonadal hormone system

Certain disorders of puberty

Polyglandular dysfunction

Disorders of lipoprotein metabolism and certain specified lipidaemias

Undernutrition - Two new subsections have been added for Undernutrition based on anthropometric or clinical criteria and Undernutrition due to specific nutrient deficiencies

Part of the grouping - Undernutrition

Overweight, obesity or specific nutrient excesses

Metabolic disorders with subsections based on aetiology

3.15.6 Differences between ICD-10 and ICD-11 in Chapter 06

Changes to this chapter include restructuring of the hierarchy, the inclusion of more current terminology, and specific groupings for single episodes of harmful use, harmful pattern of use, dependence, intoxication, and withdrawal by substance type.

In the ICD-10, the numbers of large groupings, or 'blocks', of disorders was artificially constrained by the decimal coding system used in the classification, such that it was only possible to have a maximum of ten major groupings of disorders within the mental and

behavioural disorder chapter (corresponding to the digits 0 to 9). This meant that some groupings were created that were not based on clinical utility or scientific evidence. In the ICD–10, for example, one block (F30-F39) is devoted to Mood (affective) disorders, while Anxiety disorders represent only a portion of a broad and heterogeneous block (F40- F49) called ‘Neurotic, stress-related, and somatoform disorders’. Another block ? ‘Behavioural syndromes associated with physiological disturbances and physical factors’ ? unites disorders that are unrelated in terms of clinical symptoms and symptomatology except that they have something to do with the body.

Given the constrained structural parameters of the ICD–10, the developers of the classification provided a reasonable set of diagnostic groupings. However, the more flexible structural characteristics of ICD–11 make it possible to incorporate key features based on available scientific evidence and current practice for more optimal nosology.

Table 1: Comparison of ICD-10 block structure with ICD-11 equivalent structure

ICD-10 block heading	ICD-11 equivalent structure
F00-F09 Organic, including symptomatic, mental disorders	Neurocognitive disorders
F10-F19 Mental and behavioural disorders due to psychoactive substance use	Part of the grouping - Disorders due to substance use or addictive behaviours
F20-F29 Schizophrenia, schizotypal and delusional disorders	Schizophrenia or other primary psychotic disorders
F30-F39 Mood (affective) disorders	Mood disorders
F40-F48 Neurotic, stress-related and somatoform disorders	Anxiety or fear-related disorders
F50-F59 Behavioural syndromes associated with physiological disturbances and physical factors	Redistributed between Feeding or eating disorders, Mental or behavioural disorders associated with pregnancy, childbirth and the puerperium and new chapters for Sleep disorders and Sexual health
F60-F69 Disorders of adult personality and behaviour	Personality disorders and related traits
F70-F79 Mental retardation	Part of the grouping - Neurodevelopmental disorders
F80-F89 Disorders of psychological development	Part of the grouping - Neurodevelopmental disorders
F90-F98 Behavioural and emotional disorders with onset usually occurring in childhood and adolescence	Part of the grouping - Neurodevelopmental disorders
F99-F99 Unspecified mental disorder	Unspecified residual for the chapter

3.15.7 Chapter 07 is a new addition to ICD-11 and was not found in past editions

Table 1: Comparison of ICD-10 block structure with ICD-11 equivalent structure

ICD-10 previous location	ICD-11 equivalent structure
Codes from Mental Health and Nervous system chapters	Insomnia disorders
Concept not in ICD-10	Sleep-related movement disorders
Codes from Mental Health and Nervous system chapters	Hypersomnolence disorders
Codes from Neurology and Endocrine chapters	Sleep-related breathing disorders
Codes from Mental health chapter	Parasomnia disorders
Codes from Neurology chapter	Disorders of the sleep-wake schedule
Codes from Neurology chapter	Certain specified sleep disorders

3.15.8 Differences between ICD-10 and ICD-11 in Chapter 08

There has been a major restructuring and movement of previous ICD-10 concepts in this chapter. A number of new concepts have also been added. Cerebrovascular diseases have been moved to the Neurology chapter and multiply parented to the Circulatory chapter. Transient Ischaemic attack (TIA) is now also located under Cerebrovascular diseases and appears in Diseases of the nervous system.

ICD-11 sees a major overhaul in the organisation of the blocks which make up the neurology chapter. The restrictive decimal coding system of the ICD-10, with its capacity to contain only 11 blocks of disorders per chapter, resulted in blocks containing miscellaneous neurological entities which did not logically fit together, such as the episodic and paroxysmal disorders block, containing headache disorders, epilepsy, transient ischaemic attacks and sleep disorders. The ICD-11 now positions headache disorders, epilepsy and cerebrovascular disorders at block level, and sleep disorders at chapter level (Chapter 07).

Not only has the structure of the neurological chapter changed, but the approach to classification also integrates current clinical practice and advancements in the understanding of neurological diseases. In the time since the ICD-10 was published, enormous progress in the fields of genetics, molecular biology and medical technologies have been made. An increase in the number of codes is inevitable when one reflects on the recent knowledge gain in neurology, so a balance between comprehensiveness, clinical utility and maintaining a public health approach is the aim. The working groups tackled this issue by considering the more common disorders to appear in the chapter, with less common aetiological variations of these disorders being subject to a 'double coding' technique. One major change which illustrates the advancement of knowledge is the addition of a block entitled 'Paraneoplastic and autoimmune disorders of the nervous system'. This block contains immune-mediated neurological diseases, a field in which knowledge has exploded in recent years. A second example of how the new version reflects molecular biological advancement is through awarding Prion diseases block status despite their rarity. Previously, they featured as part of the infections of the central nervous system block, but research interest after the major public health issue in Europe in the 1990s has led to new variants of prion diseases being discovered.

Table 1: Comparison of ICD-10 block structure with ICD-11 equivalent structure

ICD-10 block heading	ICD-11 equivalent structure
G00-G09 Inflammatory diseases of the nervous system	This section is now located in Chapter 1 in a new block called Non-viral infections of the central nervous system
G10-G14 Systemic atrophies primarily affecting the central nervous system	Split between the Movement disorders and Motor neuron disease and related disorders
G20-G26 Extrapyrimal and movement disorders	Movement disorders
G30-G32 Other degenerative diseases of the nervous system	Neurocognitive disorders
G40-G47 Episodic and paroxysmal disorders	Epilepsy and seizures, Headache disorders and cerebrovascular blocks. Sleep disorders are now a stand-alone chapter (Ch. 07)
G50-G59 Nerve, nerve root and plexus disorders	Disorders of nerve root, plexus and peripheral nerves
G60-G64 Polyneuropathies and other disorders of the peripheral nervous system	Polyneuropathy, Mononeuropathy and Hereditary neuropathy
G70-G73 Diseases of myoneural junction and muscle	Diseases of neuromuscular junction and muscle
G80-G83 Cerebral palsy and other paralytic syndromes	Cerebral palsy
G90-G99 Other disorders of the nervous system	Other disorders of the nervous system - the following have been moved out to their own grouping: Diseases of the autonomic nervous system, Disorders of cerebrospinal fluid pressure and flow, Spinal cord disorders excluding trauma
A81 Atypical virus infections of central nervous system	Prion diseases

3.15.9 Differences between ICD-10 and ICD-11 in Chapter 09

There have been major changes to the structure and hierarchy of this chapter for ICD-11. The aetiology/manifestation convention (dagger/asterisk) of ICD-10 has not been kept in ICD-11.

Table 1: Comparison of ICD-10 block structure with ICD-11 equivalent structure